

PACIFIC DIAGNOSTICS LAB

Clinical & Reference Laboratory Services
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CLIA #05D2174981 · CAP#88419-22

Accession PDX-25102113
Report Date: 2025-10-21 14:08 PT
Report Status: FINAL
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PDX25102113

PATIENT		ORDERING PROVIDER		SPECIMEN	
Name	CHEN, MARGARET	Name	RAO, ANJALI MD	Type	Serum, SST
DOB	1967-08-14 (58F)	NPI	1659302147	Fasting	Yes (12 hr)
MRN	MRN-2026-04481	Practice	Berkeley Family Medical Grp.	Collected	2025-10-21 07:32 PT
Sex	F			Received	2025-10-21 11:51 PT
Phone	(510) 555-0148	Account	BFMG-022	Container	Gold-top SST x2
Address	4421 Magnolia Ave, Apt 3B Berkeley, CA 94705	ICD-10	E78.5; I10; E11.9	Volume	8.0 mL
		Indication	Hyperlipidemia f/u; HTN; T2DM		

LIPID	PANEL	WITH	DIRECT	LDL	LOINC 57698-3			
TEST		RESULT	FLAG	REFERENCE RANGE		UNITS	METHOD / NOTE	
Cholesterol,	Total	268	H	Desirable Borderline High	<200 200-239 / ≥240	mg/dL	Enzymatic colorimetric	
HDL Cholesterol		46	L	Female Optimal	≥50 ≥60	mg/dL	Direct, homogeneous	
LDL Cholesterol, Calculated		178	H	Optimal Near optimal / Borderline / High	<100 100-129 130-159 160-189	mg/dL	Calculated (Friedewald)	
Triglycerides		205	H	Normal Borderline High	<150 150-199 / 200-499	mg/dL	Enzymatic, PAP	GPO-
Non-HDL Cholesterol		222	H	Goal (Total	<130 - HDL)	mg/dL	Calculated	

INTERPRETIVE COMMENTS
Mixed dyslipidemia, partial response. Compared to the 2025-07-18 panel, LDL-C has decreased from 195 to 178 mg/dL (~9% relative reduction) on moderate-intensity statin therapy. LDL-C remains in the borderline-high range and Non-HDL-C 222 mg/dL still exceeds the <130 mg/dL goal. Continued therapy, lifestyle reinforcement, and consideration of statin-intensity escalation are reasonable per the 2018 ACC/AHA Cholesterol Clinical Practice Guideline. Reference ranges per NCEP ATP III.